

إسم المريض ربيعاً : مي حمدي محمد شحاته	
المحافظة : القليوبية	الحي / المدينة : القناطر الخيرية
رقم البطاقة : 28605250105066	رقم التليفون : 01281891316
العمر(تاريخ الميلاد) : 25-05-1986	المهنة : حاصل على دبلوم
العنوان : سندبیس مرکز القناطر الخيرية - القليوبية	
تاريخ تسجيل الإستمارة : 01-10-2026	

أقر انا الموقع أدناه انني لا اتمتع بأي نظام علاج مثل:- التأمين الصحي-القوات المسلحة-الشرطة-
الهيئات-المؤسسات او أي نظام يتعارض مع نظام العلاج علي نفقة الدولة- وسوف اتحمل كافة
نفقات العلاج في حالة ثبوت غير ذلك
الإحصائي الاجتماعي
التوقيع: (-----) المقر بما فيه
التوقيع: (-----)

خطة العلاج : أبحاث أورام مزيد من الابحاث و الاشعات [7954] قسم العلاج : الاورام
التعليق على التشخيص : -----

الاسم: د/محمد كيلاني الوظيفة: _____
 الاسم: د/ندى حسن الوظيفة: _____
 الاسم: د/نرمين رمضان الوظيفة: _____

أطلعت اللجنة علي لائحة العلاج الخاصة بالمجالس
 وتلتزم جهة العلاج بلائحة المجالس الطبية.
 كما تقر اللجنة بأن البيانات المدونة في هذا النموذج صحيحة.

تعتمد صحة البيانات المرفقة
 رئيس اللجنة ومدير
 المستشفى.

تتعهد المستشفى أن لا يتم تنفيذ القرار إلا بعد مراجعة الملف الطبي للمريض وإعادة مناظرة المريض بالمستشفى قبل الصرف.

----- / د ----- / د ----- / د. | ----- / د. |

التشخيص النهائي :
 خطة العلاج :
 المستشفى :
 قسم :
 جهة العلاج :
 ملاحظات :
 نفقات :

رئيس
المجلس الطبي المتخصص

--

Name: مي حمدي محمد شحاته National ID: 28605250105066	
---	---

تقرير طبي

السيد
تحية طبية وبعد,,

إيماء للطلب المقدم من	بشأن تقرير طبي للمريض
بقسم:	رقم طبي:

نتشرف بان نوضح فيما يلي تقريرنا عن حالته
--

تاريخ وساعة الدخول : -- --:-- yyyy-mm-dd	تاريخ وساعة الخروج : PM 01:17 05-07-2026
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وبعد الكشف والفحوصات والمتابعة وجد انه
--

المریضة تعاني من ورم غير حميد بالثدى و تحتاج الي Further investigations and imaging

التاريخ : 05-07-2026	الوقت : -- --:--
	الطبيب المعالج : د/شيماء لاشين

PATIENT NAME: Mai Hamdi Mohammed Shehata

GENDER: Female

DATE : 30-Apr-2025 12:31 PM

PATIENT ID: 2019097043

AGE: 38 y

ORDER: 3352630

99MTC-MDP BONE SCAN**Desired Information:** -
skeletal survey.**Technique:**

Delayed Whole body imaging in anterior and posterior projection performed 2 hours after I.V administration of 20 mCi 99m Tc-MDP in the following manner:

Report:

No hot spot that account for osseous deposits could be detected all over the surveyed skeleton.

Mild Degenerative changes are seen over lumbar vertebrae.
Both kidneys are normally visualized.

Conclusion:

No definite evidence of metastatic osseous deposits.

Reported By,**Prof. Huda Fathy, Nuclear Medicine Consultant****01-May-2025 03:28 PM**

PATIENT: Mai Hamdi Mohammed Shehata, 2019097043, Female, 38 y

Report version 1, 01-May-2025 03:28 PM

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مركز بهية الهرم

4 شارع علوية أول شارع الهرم بجوار داري , الجزيرة



مستشفى بهية الشيخ زايد

٤٠ الجي الأول - الشيخ زايد لأم كلية الهندسة

Bone Scan

[[DOCTYPE:BONE_SCAN]]



ImmunohistoChemistry report

Report Number: m4822

مرضى :	مي حمدي محمد شحاته	نوع :	Female
ن :	34/Y	العنوان :	السلام
تاريخ اخذ العينة :	28/08/2019 14:04	تاريخ استلام العينة :	28/08/2019 14:04
ب التحليل :	مركز اورام دار السلام	تاريخ التقرير :	03/09/2019 15:02
بإية :	مركز اورام دار السلام		

Primary Sample :

Paraffin Block

Test :

ER & PR & HER2 neu (ER & RP : Harvey scoring system - Her2neu : CAP scoring system)

Comment :

Technique :

sample recieved was treated by : - CONFIRM anti-estrogen (ER) (SP1) Rabbit monoclonal primary antibody.- CONFIRM anti-progesterone receptor (PR) (IE2) Rabbit monoclonal primary antibody.

Sample recieved was treated by : - VENTANA anti-HER2/neu (4B5) Rabbit monoclonal primary antibody using DAB Detection kit in ventana benchmark GX.

Result :

ER: moderate nuclear staining of 80 % of tumour cells
PR: moderate nuclear staining of 80% of tumour cells
HER2/neu protein over expression: discontinuous membrane staining of invasive tumor cells.

Referral Lab :

Comment :

Conclusion :

ER: POSITIVE (SCORE 7/8)

PR: POSITIVE (SCORE 7 /8)

HER2/NEU PROTEIN OVER EXPRESSION : NEGATIVE (SCORE 1+) OF INVASIVE TUMOUR CELLS (CORE BIOPSY)

Pathologist:

د/نبيل رزقي

Lab Director:

د/توفيق ماهر



جمهورية مصر العربية
بطاقة تحقيق الشخصية

مي
حمدي محمد شحاته

سندبيس
مركز القناطر الخيرية - القليوبية

١٩٨٦/٠٤/٢٥

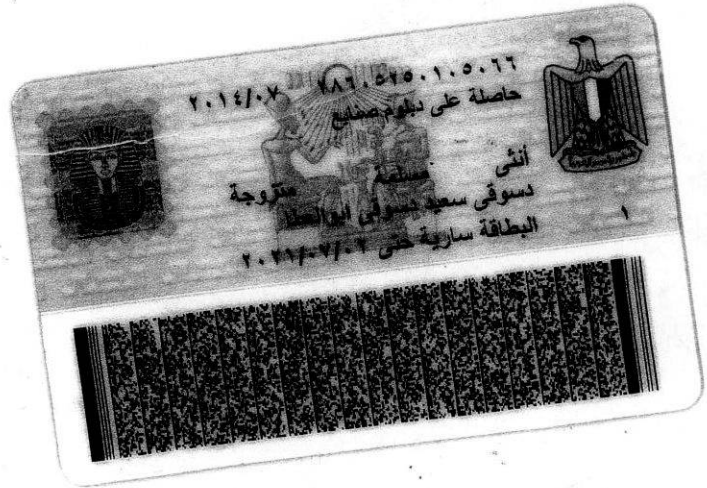
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FS5115591



National ID - Front

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National ID - Back

[[DOCTYPE:ID_BACK]]



Pathology Report

Name: مى حمدى محمد شحاته

Age: 33 years.

Clinical data: Left breast mass.

Specimen type: Tru cut biopsy.

Date: 19/8/2019

Lab. Number: 701

Hospital Number: 865/2019

Gross Description:

Multiple soft tan admixed with yellow cores, the longest is 1.5 cm, totally submitted.

Microscopic Examination:

Section examined revealed cores of breast tissue showing invasive files and tubules of moderately anaplastic ductal epithelial cells, associated with 10% DCIS, cribriform pattern and fair desmoplastic stroma.

Diagnosis:

Left breast mass, core biopsy;

Invasive duct carcinoma GII with about 10% DCIS for ER, PR, HER2 and KI67).

Prof. Dr. Amal Haridy

Prof. Dr. Essam Ayad



Pathology Report

Name: مي حمدي محمد

Age: 34 years.

Clinical data: Left breast, Post MRM, complicated cyst

Specimen type: US guided Aspiration.

Date (Received): 24/4/2021

Date (reported): 4/4/2021

Lab. Number: C- 84/21

Hospital Number: 3284/19 نفقة

Gross Description:

13 ml of turbid yellowish fluid.

Microscopic Examination:

Cytological examination of the smears prepared revealed hypocellular smears consistent with mild Inflammatory smears, free from cytological malignancy

Diagnosis:

Left breast, Post MRM, complicated cyst, US guided aspiration;

Hypocellular smears consistent with mild Inflammatory smears, free from cytological malignancy

Prof. Dr. Amal Hareedy.

Dr. Marwa Mahmoud



Pathology Report

Name: مي حمدي محمد شحاته

Age: 34 years.

Clinical data: Left breast mass, post chemotherapy.

Specimen type: Skin sparing mastectomy & axillary clearance.

Date: 14/3/2020

Lab. Number: 479

Hospital Number: 3284/2019

Gross Description:

Skin sparing mastectomy specimen; 20x15x4 cm, including nipple and areola, attached axilla; 5x4 cm. Serial cuts revealed an ill-defined firm greyish white mass measured 1.2x1 cm, located in the lower outer quadrant of the breast about; 5 cm away from nipple with least resection margin is the deep margin; 2.5 cm away.

Rest of breast showed rubbery greyish white areas, admixed with mammary fat.

Axillary dissection revealed 9 nodules, up to 1.7x1.5 cm.

Microscopic Examination & Diagnosis:

Left breast mass, post chemotherapy, skin sparing mastectomy & axillary clearance;

- Multicentric invasive duct carcinoma GII.
- 10% DCIS.
- Minimal necrosis.
- Free nipple, areola & resection margins.
- Free axillary lymph nodes 0/19.
- Tumor stage ypT1 ypN0.

~~Prof. Dr. Amal Haridy~~

~~Prof. Dr. Essam Ayad~~

Pathology Report

Name: مري حدى محمد شحاته

Age: 33 years.

Clinical data: Left breast mass.

Specimen type: Tru cut biopsy.

Date: 19/8/2019

Lab. Number: 701

Hospital Number: 865/2019

Gross Description:

Multiple soft tan admixed with yellow cores, the longest is 1.5 cm, totally submitted.

Microscopic Examination:

Section examined revealed cores of breast tissue showing invasive files and tubules of moderately anaplastic ductal epithelial cells, associated with 10% DCIS, cribriform pattern and fair desmoplastic stroma.

Diagnosis:

Left breast mass, core biopsy;

Invasive duct carcinoma GII with about 10% DCIS for ER, PR, HER2 and KI67).

~~Prof. Dr. Amal Haridy~~

~~Prof. Dr. Essam Ayad~~



Patient Name: Mai Hamdy Mohamed Shehata	Date : 22/01/2024
Exam : MAMMOGRAPHY BOTH	Age : 37 Year(s)
Medical ID: 3284/2019	Pat. ID: 28546

RIGHT BREAST MAMMOGRAPHY WITH BILATERAL US

Indication:

Patient had left MRM with muscle flap reconstruction

Technique

- right breast MLO and CC views
- Bilateral ultrasound

Report:

- The right breast is scattered fibroglandular densities of the right breast (ACR B)
- No spiculated masses or grouped microcalcifications.
- Complementary ultrasound also showed no right breast solid or cystic masses
- Normal skin thickness and nipple-areola complex, with average caliber of the retro-areolar ducts
- Nonspecific right axillary LN, with preserved fatty hila and uniform cortical thickness.
- Free left operative bed with no evidence of recurrent mass lesions, apart from stationary LIQ dystrophic macrocalcifications, with no underlying suspicious findings seen by US
- Free left infra and supra clavicular regions.

Opinion:

- Left MRM with free operative bed with no evidence of recurrent mass lesions
- No right breast suspicious findings (BIRADS 2)

Yours truly

Signature Dr. Yasmin Nada

22/01/2024 12:43 PM



Patient Name: Mai Hamdy Mohamed Shehata	Date : 11/05/2025
Exam : MAMMOGRAPHY BOTH	Age : 39 Year(s)
Medical ID: 3284/2019	Pat. ID: 28546

RIGHT BREAST MAMMOGRAPHY WITH BILATERAL US

Indication:

Patient had left MRM with muscle flap breast reconstruction

Technique

- BOTH breast MLO and CC views
- Bilateral ultrasound

Report:

- The right), scattered fibroglandular densities of the right breast (ACR B)
- No spiculated masses or grouped microcalcifications.
- Complementary ultrasound also showed no right breast solid or cystic masses
- Normal skin thickness and nipple-areola complex, with average caliber of the retro-areolar ducts
- Nonspecific right axillary LN, with preserved fatty hila and uniform cortical thickness.
- Free left operative bed with no evidence of recurrent mass lesions with post operative dystrophic calcification and oil cyst.
- Free left: infra and supra clavicular regions.

Opinion:

- Left MRM with muscle flap breast reconstruction with free operative bed with no evidence of recurrent mass lesions BIRADS II
- No right breast suspicious findings (BIRADS I)

DR REHAB EL BADRI

Signature Dr . Rehab El Badry

11/05/2025 2:05 PM

PATIENT NAME: Mai Hamci Mohammed Shehata

GENDER: Female

DATE : 30-Apr-2025 12:31 PM

PATIENT ID: 2019097043

AGE: 38 y

ORDER: 3352630

99MTC-MDP BONE SCAN

Desired Information: -
skeletal survey.

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Report:

No hot spot that account for osseous deposits could be detected all over the surveyed skeleton.

Mild Degenerative changes are seen over lumbar vertebrae.

Both kidneys are normally visualized.

Conclusion:

No definite evidence of metastatic osseous deposits.

Reported By,

Prof. Huda Fathy, Nuclear Medicine Consultant

01-May-2025 03:28 PM

PATIENT: Mai Hamdi Mohammed Shehata, 2019097043, Female, 38 y

Report version 1, 01-May-2025 03:28 PM

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مركز دمية البرم
ه شارع علوية أول شارع الهرم بجوار داري , الجيزة

مستشفى بهية الشيخ زايد
٤٠ النجى الأول - الشيخ زايد أمام كلية الهندسة



Mammography Report

الاسم: مي حمدي محمد
تاريخ: ٢٠١٩/٧/٢١

Bilateral Digital Mammogram

- ♦ Normal skin, nipple and subcutaneous tissue.
- ♦ Heterogeneous dense fibroglandular breast parenchyma ACRC.
- ♦ Left UOQ area of architectural distortion.

Opinion:

- Left UOQ asymmetrical distortion for breast US.
- BIRADS (0).

Signature
Fatma Salah El Din, MD.
Consultant of Radiology.

Refernces of BIRADS Classification:

BIRADS classification of breast lesions according to the American collage of radiology used for breast assessment combined with recommendations:		
Category & recommendations		Suspected Cancer Risk
Category (0)	Non-informative study..	
Category (1)	Normal study, routing screening.	0%
Category (2)	Benign findings, routing screening.	0%
Category (3)	Probably benign, for close follow up.	2%
Category (4)	Probably malignant, for biopsy.	50%
	A: mild suspicion	
	B: moderate suspicion	
	C: high suspicion.	
Category (5)	Malignant, for biopsy.	98-100%
Category (6)	Pathological proven malignant lesion.	

Name: Mrs. May hamdin mohamed

Date : 1 April 2021

Trans-vaginal ultrasound scanning revealed: -

○ **Uterus: -**

- AVF, average sized uterus for age measuring about:
10x54x45 cm in its longitudinal, transverse & AP diameters respectively.
- Endometrial thickness measures about: 4 mm, contraceptive device seen insitu.
- No gross myometrial focal lesions

○ **Ovaries: -**

- The right ovary is average in size and echopattern.
- The left ovary shows multiple simple ovarian cysts , the largest is 34mm
- No free fluid in the Douglas pouch.

Conclusion:

Left ovarian simple cysts.

Best regards

Dr/Daaa Ahmed , FEBR, MD.



Name : MAY HAMDY

Date : 16-11-2019

**POST CONTRAST MULTI-SLICE CT SCAN OF THE CHEST,
ABDOMEN & PELVIS WITH CORONAL RECONSTRUCTION: -**

Chest:

- Few tiny bilateral millimetric pulmonary nodules are noted.
- Clear both lung fields. No sizable pulmonary nodules, masses, areas of consolidation or cavitary lesions.
- No hilar or mediastinal lymph nodes.
- No evidence of pleural effusion, pleural thickening or masses seen.
- Normal cardiac size and shape with no gross abnormality of the cardiac chambers. No pericardial effusion detected.
- Normal CT appearance of the thoracic aorta and the great vessels.

Abdomen:

- The liver is enlarged in size with homogenous low CT texture. No focal lesions or contour irregularities.
- Patent homogeneously enhancing portal vein and its main branches as well as its main tributaries.
- No intra-hepatic or extra-hepatic biliary radicles dilatation.
- Normal size & parenchymal texture of the spleen with no focal lesions seen.
- Normal size, parenchymal thickness & density of both kidneys with no stones or back-pressure changes.
- Normal appearance of the pancreas, aorta, I.V.C and both suprarenals.
- Smooth filling of the urinary bladder with no filling defects or diverticular outpouching.
- Bulky uterus with IUD seen inside.
- Reviewing skeletal parts revealed bilateral ilitis condensan.

OPINION:

- ***Few tiny bilateral millimetric benign looking pulmonary nodules.***
- ***Fatty hepatomegaly with no focal lesions.***
- ***No evidence of lytic or sclerotic osseous lesions.***

DR. Ibrahim Elsedawy M.Sc

Dr.m.gomaa,md



Patient Name: Mai Hamdy Mohamed Shehata	Date : 09/02/2023
Exam : MAMMOGRAPHY BOTH	Age : 36 Year(s)
Medical ID: 3284/2019	

**BILATERAL BREAST MAMMOGRAPHY AND HIGH
RESOLUTION ULTRASOUND**

Indication:

Left breast mastectomy with reconstruction

Technique

- Bilateral MLO and CC views
- Bilateral breast ultrasound

Report:

- Scattered fibro glandular density (ACR B)

Left breast:

- LIQ well circumscribed lucent mass seen by US as 8O'clock as well circumscribed oval shaped complex solid and cystic mass , measured about 1.7x2.0 cm . minimal vascularity detected inside the solid part
- No sizable collection
- Diffuse edema pattern in the form of increased trabeculations and skin thickness.
- No regional lymphadenopathy

Right Breast:

- No suspicious micro calcifications or architectural distortion detected.
- No spiculated mass
- Normal retro-areola ducts
- Normal skin thickness, and nipple-areola complex
- Nonspecific axillary LN.

Opinion:

- Post MRM status showing 8 fat necrosis , however regarding internal vascularity ,
- Normal study of the right breast (BIRADS 1)

Best regards,
Dr/Asmaa Zeinhom, Msc

Signature Temp

09/02/2023 2:37 PM

Name: may hamdy mohamed
Date: 1 April 2021

ABDOMINO-PELVIC ULTRASOUND EXAMINATION REVEALED:

Liver: -

- enlarged in size (about 19 cm at the right MCL), smooth outline, bright echopattern.
- No intra hepatic biliary radicle dilatation.
- Portal vein is not dilated and patent .

Gall Bladder: -

- Average size , , normal wall thickness, no stones.
- CBD is not dilated.

Spleen: -

- Average size (About 10 cm), displaying homogenous echopattern.

Kidneys: -

Are of normal size , site , well differentiated & with no gross stones or backpressure changes. no solid or cystic lesion.
-Para-aortic and pancreatic area: no gross lesion
No ascites.

Urinary Bladder: -

- Average capacity with no stones, masses or diverticular out pouching

Uterus : average in size , normal endometrial line , no focal lesion.

Both ovaries : the right ovary is average in size and echopattern , the left ovary shows multiple simple cysts , (the largest is measuring 35).

Opinion:

- Bright enlarged liver (fatty)..
- Left simple ovarian cysts (likely functional for follow up.

Best regards

Dr: Doga Ahmed . FEBR, MD



Ministry of health
Central health laboratories
Pathology department

ImmunohistoChemistry report

Report Number: m4822

Female	النوع	مي حمدي محمد شحاته	اسم المريض
السلام	العنوان	340Y	السن
28/08/2019 14:04	تاريخ استلام العينة		تاريخ اخذ العينة
03/09/2019 15:02	تاريخ التقرير	مركز اورام دار السلام	طالب التحليل
		مركز اورام دار السلام	عناية

Primary Sample :

Paraffin Block

Test :

ER & PR & HER2 neu (ER&RP : Harvey scoring system - Her2 neu : CAP scoring system)

Comment :

Technique :

sample recieved was treated by: - CONFIRM anti-estrogen (ER) (SP1) Rabbit monoclonal primary antibody.- CONFIRM anti-progesterone receptor(PR) (IE2)Rabbit monoclonal primary antibody.
Sample recieved was treated by: -VENTANA anti-HER2/neu (4B5) Rabbit monoclonal primary antibody using DAB Detection kit in ventana benchmark GX.

Result :

ER:moderate nuclear staining of 80 % of tumour cells
PR: moderate nuclear staining of 80% of tumour cel's
HER2/neu protein over expression :discontinuous membrane staining of invasive tumor cells.

Referral Lab :

Comment :

Conclusion :

ER: POSITIVE (SCORE 7/8)
PR: POSITIVE (SCORE 7/8)
HER2/NEU PROTEIN OVER EXPRESSION : NEGATIVE (SCORE 1+) OF INVASIVE TUMOUR CELLS (CORE BIOPSY)

Pathologist:

د/أيناس رمزي

Lab Director: د/ نريزا ماهر

19 El-sheikh Rehan Street, Abeideen, Cairo
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يتم الاحتفاظ بأصل العينة لمدة 30 يوم من تاريخ استلامها